

Postpartum - C-section Symptoms - Female (Child)

Algorithm 1432 | Child (12-17 years) | Gender at birth: Female

Definition

- C-section recovery symptom assessment decomposed from NHS.UK's published C-section recovery guidance.
- Child patient.
- Female.

Initial assessment

1. [DURATION] How many days since the C-section?
2. [OPEN_TEXT] Describe how the wound looks.
3. [YES_NO] Any fever, heavy bleeding, leg pain, chest pain, or breathing difficulty?
4. [YES_NO] If the patient is under 18, can they speak privately now, and do they say they feel safe with the accompanying adult?
5. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Redirect → Skin Injury

Is this a skin wound or injury unrelated to a recent C-section?

A non-operative skin wound or injury belongs in the Skin Injury pathway.

1. Disposition level 100

Is there trouble breathing, chest pain, fainting, heavy or gushing vaginal bleeding, confusion, or rapidly worsening severe pain or illness?

Breathing difficulty, chest pain, fainting, heavy bleeding, confusion, or rapidly worsening illness are urgent maternal warning signs requiring immediate emergency care.

1. Disposition level 70

Is the wound increasingly red, painful, swollen, opening, or draining pus or foul-smelling fluid; is there fever, persistent severe incision pain, painful urination, or leaking urine without the emergency features above?

NHS.UK guidance lists wound infection and urinary symptoms after a C-section as requiring prompt clinical review.

1. Disposition level 15

Is this normal, expected C-section recovery with none of the features above?

NHS.UK guidance describes routine wound care and gradual activity increase as the expected recovery path.

Advice

Emergency C-section complication precautions

- Call Qatar emergency services on 999 now. Do not drive yourself. Tell the call handler about the recent C-section. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: worsening pain or bleeding; breathing difficulty.

Urgent C-section complication review

- Arrange prompt clinical assessment through the locally approved Qatar maternity pathway; the exact non-emergency destination remains GOVERNANCE_REQUIRED.
- Call Back If: symptoms worsen; fever develops.

Home care for normal C-section recovery

- Follow the discharge team's wound-care and activity instructions. Keep the wound clean and dry, use sanitary pads rather than tampons, and increase gentle activity gradually. A clinician or pharmacist should confirm any pain medicine is suitable for this patient, including during breastfeeding.
- Call Back If: wound becomes red, painful, swollen, opens, or drains fluid; fever, heavy bleeding, chest pain, breathing difficulty, or one-sided leg pain develops.

First aid

- Call Qatar emergency services on 999 now. Do not drive yourself. Tell the call handler about the recent C-section. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Caesarean section - Recovery", <https://www.nhs.uk/conditions/caesarean-section/recovery/> (page last reviewed 04 January 2023)

Provenance

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redirect. Demographic variants contain only source-supported differences; identical thresholds are documented rather than fabricated. Local destinations and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

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